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PW **ABSTRACT**

Reimagining T2D Self-Management Programs to Address Cost & Contributor Access Barriers

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Proper management of type 2 diabetes (T2D) encompasses all aspects of a person's lifestyle. Self-management education and support are critical components of effective care.

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Although resources are available to address this, specific populations, such as Asian American communities, are impeded from accessing these support systems because of cultural stigma and a lack of resources tailored to their experiences. To address this gap in the care system, Dante Anthony Tolentino, PhD, RN, and colleagues conducted a qualitative descriptive study, using semistructured interviews to explore the experiences of Asian American patients with T2D.

The results were published in *The Science of Diabetes Self-Management and Care*. Dr. Tolentino spoke to *Physician's Weekly* about the impact of their findings.

PW: Why are diabetes self-management education and support essential components of optimal care?

Dr. Tolentino: Diabetes is a complex and challenging disease that requires daily self-management decisions by the person living with it. Diabetes self-management education and support (DSMES) addresses the comprehensive blend of clinical, educational, psychosocial, and behavioral aspects of care needed for daily self-management.

A key purpose of DSMES is to equip individuals with the knowledge, skills, and confidence to take responsibility for their self-management, which includes collaborating with their healthcare team, making informed decisions, solving

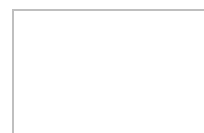
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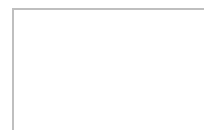
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problems, setting personal goals, developing action plans, and managing emotions and life stresses. This facilitated knowledge and skill development leads to sustainable behavioral changes that can improve diabetes outcomes.

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Individuals who have received DSMES services demonstrate enhanced self efficacy and empowerment. Effective diabetes self-management, which DSMES supports, has been shown to lead to significant clinical benefits. These include improved hemoglobin A1c (A1C) with reductions that are additive to lifestyle and drug therapy, resulting in an average A1C reduction compared to usual care.



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What makes this issue urgent in the healthcare landscape?

The urgency stems from the significant and disproportionate burden of T2D and its complications on Asian Americans, coupled with the proven effectiveness of DSMES in reducing these risks, which is offset by the critically low DSMES engagement within this population due to barriers like lack of awareness and accessibility. Bridging this gap is crucial for addressing the health disparities faced by Asian Americans with T2D.

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What are the main barriers to accessing DSMES programs for this patient population?

Our study highlights critical barriers preventing Asian Americans with T2D from accessing DSMES programs. A compelling finding is a significant lack of awareness regarding the existence of DSMES programs, with most participants reporting they were not informed about these services, often by their healthcare providers.

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Compounding this issue, participants frequently found traditional DSMES content lacking in cultural relevance and tailoring, particularly concerning dietary guidance aligned with Asian food practices, leading them to seek information from informal sources such as family, friends, and online platforms, including social media. The study also revealed that cultural stigma and shame surrounding diabetes significantly impact engagement, making individuals reluctant to disclose their condition and less likely to seek support.

Beyond cultural and informational gaps, structural barriers such as cost and limited accessibility of programs hinder participation. These findings highlight an urgent need for healthcare providers to proactively discuss and refer patients to DSMES and for programs to become more culturally responsive, accessible, and tailored to effectively support Asian Americans in managing T2D and preventing devastating complications.

How else might your findings impact clinical practice for patients with T2D?

Understanding this disparity underscores the need for tailored interventions for Asian Americans to mitigate diabetes complications. The finding that most participants were unaware of DSMES and were not informed by their providers points to a critical gap in healthcare delivery. This highlights the need for practice to emphasize proactive provider education and standardized referral protocols for DSMES at critical junctures.

The study also reinforces that barriers like cost, accessibility, scheduling conflicts, and format preferences significantly

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hinder participation. Practices can benefit from offering diverse and flexible DSMES formats, including virtual or online options, shorter sessions, and convenient locations. Advocating for and facilitating reimbursement processes that reduce patients' financial burden is also crucial.

Recognizing that Asian Americans often rely on informal networks (such as family and friends) and online sources (including social media) for diabetes information can inform practice strategies. Rather than viewing these solely as alternatives to formal DSMES, practices can integrate evidence-based, culturally tailored information into these preferred channels, for example, by leveraging social media. DSMES programs can also work to involve family members and guide patients toward credible online resources.

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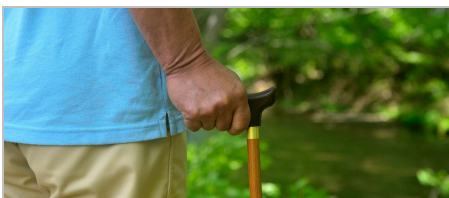
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